Raleigh Radiology Blue Ridge
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Name:	Swethashree Anbazhagan	Exam Date:	5/13/2020 03:28 PM
DOB:	10/15/1994	Exam Name:	XR ABDOMEN 1 VIEW | 74018
Phone:	(984)269-8705	Referrer:	Jaimita Patel, MD
Patient ID:	1119222	Referrer #1 Phone:	9198750539
Acc #:	3017312		
SID:	3017312		

ABDOMEN ONE VIEW:

HISTORY: Left lower quadrant pain for 6 months.

COMPARISON: None

TECHNIQUE: Single view of the abdomen

FINDINGS: The bowel gas pattern is normal. No localized transition zone or dilated bowel loops to suggest obstruction. There are no abnormal calculi visualized over the kidneys, ureters, or urinary bladder. Normal volume of stool is present through all colonic segments.

IMPRESSION: Negative single view of the abdomen.

https://mobility.raleighrad.com:8443/app?name=imageviewer&client=flex&AccessionNumber=3017312

Report Electronically Signed by: Daniel Shekleton
Report Electronically Signed on: 5/13/2020 06:13 PM

Completed Date:	5/13/2020 03:28 PM	Read Location:	POS1203
Technologist:	Kris Teixeira	Interpreting Rad:	Daniel Shekleton
		Dictated Date:	5/13/2020 06:13 PM

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Name:	Swethashree Anbazhagan	Exam Date:	6/3/2020 09:24 AM
DOB:	10/15/1994	Exam Name:	CT ABDOMEN AND PELVIS WITH CONTRAST | 74177
Phone:	(984)269-8705		
Patient ID:	1119222	Referrer:	Jaimita Patel, MD
Acc #:	3027194	Referrer #1 Phone:	9198750539
SID:	3027194		

CT ABDOMEN AND PELVIS WITH CONTRAST:

HISTORY: Left lower quadrant pain. 100cc Omnipaque 300.

COMPARISON: X-ray dated 5/13/2020

CONTRAST:
Type of IV contrast: Omnipaque 300
Contrast injected: 100 ml
Contrast wasted: 0 ml

TECHNIQUE: Spiral CT performed from the dome of the diaphragm to the pubic symphysis following IV contrast.

Dose reduction technique was used on this scan by utilizing automated exposure control, adjustment of the mA and/or kV according to the patient size. DICOM format image data available to non-affiliated external healthcare facilities or entities on a secure, media free, reciprocally searchable basis with patient authorization for at least a 12 month period after the study.

FINDINGS:

Scout: No additional findings

Lung bases: No significant opacities or pleural fluid.

Hepatobiliary: Hypodensity right hepatic lobe, too small to characterize. Gallbladder is unremarkable. Pancreas normal morphology and duct caliber. Normal splenic parenchyma.

GU: Bilateral symmetrical renal parenchymal enhancement, without solid mass, hydronephrosis or nephrolithiasis. Normal ureteral caliber. No abnormality identified in the bladder. Normal adrenal morphology. Complex right adnexal lesion measuring 7.4 x 7.6 x 5.9 cm with hyperdense and hypodense components exerting mass effect upon the uterus, displacing it to the left. Left adnexa is grossly unremarkable.

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GI: No evidence for bowel obstruction. No mesenteric stranding or wall thickening. No intraperitoneal adenopathy, masses or fluid. No evidence of acute appendicitis.

Retroperitoneum: No retroperitoneal adenopathy.

Cardiovascular: Normal aortic caliber.

Musculoskeletal: No focal lytic or blastic lesion. No fracture or acute derangement. No acute osseous pathology.

IMPRESSION:
Complex right adnexal lesion measuring 7.4 x 7.6 x 5.9 cm with hyperdense and hypodense components exerting mass effect upon the uterus, displacing it to the left, possibly reflecting a hemorrhagic cyst or endometrioma. Lesion size may predispose to ovarian torsion. Recommend pelvic ultrasound for further evaluation.

MANAGEMENT OF OVARIAN CYSTS detected on CT or MRI in women without symptoms attributable to the cyst (2019 JACR):

Simple Cysts
Premenopausal
- Well characterized simple cyst:
- <= 5cm: No follow-up imaging
- >5 to 10cm: Follow-up US in 6-12 months
- Limited assessment simple cyst:
- <= 5cm: No follow-up imaging
- >5cm to 10cm: Ultrasound in a few days.

Postmenopausal
- Well characterized simple cyst:
- <= 3cm: No follow-up imaging
- >3 to 10cm: Follow-up US in 6-12 months
- Limited assessment simple cyst:
- <= 3cm: No follow-up imaging
- >3cm to 10cm: Ultrasound in a few days.

Continued...

Name: Swethashree Anbazhagan **Patient ID:** 1119222
Date of Birth: 10/15/1994 **Acc #:** 3027194

Cyst with uncertain diagnosis: Ultrasound or MRI of the pelvis without and with contrast in a few days.

Simple cyst greater than or equal to 10 cm: MRI without and with contrast in a few days.

Hemorrhagic cyst:
In a premenopausal patient, consider ultrasound in 2-3 months if the cyst is larger than 5 cm, otherwise no further imaging.
In postmenopausal patient, consider ultrasound or MRI without and with contrast in a few days.

Note: The recommendations are offered as general guidance and do not necessarily apply to all patients. If prior imaging is available, and the cyst has been stable for 2+ year, it can be considered benign and no follow-up in needed.
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Report Electronically Signed by: Kesha Shah, MD
Report Electronically Signed on: 6/3/2020 09:40 AM

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Technologist: Cindy Loy

Read Location: POS2000
Interpreting Rad: Kesha Shah, MD
Dictated Date: 6/3/2020 09:27 AM